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Professional Credentials Exchange
Verified Data. Trusted Networks. Better Healthcare.
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June 13, 2025

Mehmet Oz, MD
Administrator, Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
7500 Security Boulevard
Baltimore, MD 21244

Thomas Keane, M.D.
Assistant Secretary for Technology Policy (ASTP)
Office of the National Coordinator for Health Information Technology (ONC)
U.S. Department of Health and Human Services
330 C St SW, Floor 7
Washington, DC 20201

Submitted via www.regulations.gov

RE: Request for Information; Health Technology Ecosystem [CMS-0042-NC]

Dear Drs. Oz and Keane:

Professional Credentials Exchange applauds CMS' interest in using technology to transform the healthcare ecosystem. We strongly support the goal of unlocking the power of modern technology to help individuals take control of their health and access care more efficiently. Our business model directly aligns with CMS' commitment to ensure health information flows to where it is needed, rather than being locked away in silos. We appreciate the opportunity to provide our perspective on this issue as an expert in the safe sharing of verified practitioner data.

Professional Credentials Exchange has developed technology infrastructure that enables the creation of verified practitioner data networks. The platform allows provider organizations, networks, and payers to securely share primary source verified (PSV) credentialing data across institutions—streamlining credentialing, enrollment, and privileging workflows, improving data accuracy, and accelerating revenue cycle operations. By reducing administrative delays and ensuring that qualified clinicians are credentialed and enrolled without unnecessary friction, the platform helps strengthen workforce readiness, enhance patient safety, and improve timely access to care. PCX has supported state Medicaid agencies, managed services organizations, and other enterprise healthcare entities in managing verified credentials data to support compliance, streamline core processes, and ensure network adequacy. The principal focus today is enabling third-party networks through strategic partnerships—providing scalable, standards-based infrastructure to support governance, interoperability, and the creation of a federated national practitioner directory. To achieve enhanced data interoperability, we urge CMS and ASTP/ONC to coordinate the development of a federated, standards-based provider directory, where accuracy improvement is the starting point, and a multipurpose, reusable network is the long-term outcome. Currently, the U.S. healthcare system faces a fundamental infrastructure gap: no shared system exists to

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verify, maintain, and distribute accurate provider data across payers, providers, and regulatory agencies. As a result:

Failure Mode	Evidence	Impact
Directory inaccuracies	48.7% of MA directory entries contain errors (CMS audit)	No Surprises Act penalties, patient abrasion, call center costs, network adequacy challenges
Credentialing delays	Avg. 90–120 days; $7,000–$8,000 per provider (Verity, MGMA)	Delayed provider onboarding, restricted access, missed revenue
Privileging gaps	29% of hospital files found incomplete (OIG)	Patient safety risk, compliance liability

These issues are costly, repetitive, and structurally unresolved—yet they all stem from the same root cause: each health care stakeholder builds and maintains its own version of a provider record, often with incomplete or outdated information.

As a health technology vendor, our experience confirms that source-verified data can be shared safely, securely, and at scale. This can lead to immediate improvements in provider directory accuracy and can enable broader administrative and clinical transformation. Our secure credentials exchange network safely shares verified practitioner data from primary sources such as licensing boards, hospitals, and payers. The integrity and fluidity of this data means it can be published once and then reused many times across the ecosystem. CMS should adopt this approach, as it supports its goal of creating a modern healthcare system that enables more personalized, timely, and accessible care.

Using Professional Credentials Exchange's expertise in secure sharing of practitioner data, we offer the following recommendations in response to RFI question PA-4:

What would be the value to payers of a nationwide provider directory that included FHIR endpoints and used digital identity credentials?

1. **Move beyond digital identity to include digital credentials.** The true value of a nationwide provider directory lies not simply in its breadth or inclusion of FHIR endpoints, but in the trustworthiness, completeness, and ongoing verification of its data. A functional provider directory that serves as a single source of provider information requires a continuously verified representation of what a provider is authorized and qualified to do, including licensure, privileges, specialties, network participation, and location data. While Login.gov, ID.me, and similar identity platforms confirm who is logging in, a directory built on verified digital credentials enables CMS to achieve broader interoperability goals. CMS should require the adoption and exchange of digitally signed, primary source verified credentials as a foundational layer of any national provider directory initiative—ensuring that directories not only identify who a provider is but reliably represent what that provider is authorized and qualified to do.

2. **Build directories on source-verified data, not self-attested updates.** Directory errors are not caused by missing FHIR endpoints. They result from disconnected, outdated, or self-reported attributes that cannot be easily reconciled across organizations. Solving this requires a network of authoritative data issuers—not a single centralized database—and a way to distribute verified data programmatically. In its efforts to create a national provider directory, CMS should require that all directory data elements subject to regulatory oversight—such as licensure, certification, and network participation—be sourced from and digitally attested by the authoritative organizations responsible for issuing or governing those attributes, using

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interoperable, machine-readable formats,, ensuring that erroneous data is not reported and repeated throughout the healthcare system.

3. **Directory accuracy is the catalyst for broader provider data reform.** Fixing directory accuracy creates the policy urgency and stakeholder alignment necessary to build a shared data network. Once that network exists, CMS should use the infrastructure to:

    - Accelerate credentialing by eliminating duplicative primary source verification
    - Streamline payer enrollment by supplying consistent, trusted provider information across contracts
    - Enhance privileging and scope-of-practice validation using source-issued attestations
    - Support compliance with the No Surprises Act, Medicare Advantage requirements, Affordable Care Act Qualified Health Plan requirements, and other delegated oversight obligations

We urge CMS to support a federated, standards-based model where directory improvement is the starting point, and a multipurpose, reusable network is the long-term outcome. Professional Credentials Exchange believes the nationwide directory should serve as the anchor use case for building the infrastructure that will ultimately transform provider data in U.S. healthcare.

Given our expertise, we respectfully request inclusion in any pilot programs or technical working groups related to:

- The creation or governance of a national provider directory
- Use of verified practitioner data in credentialing, enrollment, and compliance
- TEFCA-aligned data exchange and trust frameworks

Thank you for consideration of Professional Credentials Exchange feedback and recommendations. We would welcome the opportunity to support CMS in its efforts to establish a foundational layer of provider data infrastructure.

Please contact Leslie B. Wilkinson, jr. at lwilkinson@hashedhealth.com with any questions or to discuss our recommendations further.

Regards,

Signed by:

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Leslie B. Wilkinson, Jr., President
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